University of Iowa Health Care
Name: Yahya Mohamed Halim | DOB: 3/8/2003 | MRN: 16160482 | PCP: Hussain Banu, MD | Legal Name: Yahya Mohamed Halim

Office Visit - Feb 28, 2022

with Chris Haak at Center for Disabilities and Development - Psychology

Notes from Care Team

Consult Notes

Chris Haak at 2/28/2022 1:00 PM

Consult Orders
1. OUTPATIENT CONSULT CDD (CENTER FOR DISABILITIES AND DEVELOPMENT) [392936586] ordered by Leyser, Marcio, MD at 12/29/21 1603

Neuropsychological Evaluation
Confidential
THIS REPORT IS NOT TO BE RELEASED WITHOUT THE EXPRESSED WRITTEN CONSENT OF THE PARENT OR GUARDIAN

Name: Yahya Halim	**Dates of Evaluation:** 2/28/2022 and 3/17/2022
Date of Birth: 3/8/2003	**Age:** 19 y.o. (18 at the time of evaluation)
School: Fairfield High School	**Grade**: 12th
Preferred Language: English	**Examiner:** Christopher Haak, Ph.D.

Reasons for Referral
Yahya is a 19-year-old, bilingual (Arabic and English) male who was referred for a neuropsychological evaluation. Current concerns include cognitive, social, and emotional functioning. The evaluation consisted of review of medical and school records, review of emotional/behavioral questionnaires, clinical interview with Yahya and his mother and administration of a selected battery of neuropsychological measures.

Summary
Please note Yahya was raised in Egypt and Malaysia for much of his childhood and teen years. He started learning English at 7 years old and is fluent enough in English to attend school in Iowa and to complete the administered battery. However, his primary language spoken while living abroad was Arabic, and he continues to primarily use Arabic when speaking with his family at home. Available normative samples are composed of individuals born and raised in the US with English as their first language. Given the discrepancy between Yahya and these sample, some caution should be utilized when interpreting these results.

However, sufficient evidence from collateral sources and self report of daily functioning exists to suggest that he exhibits significant delays in his current functioning.

Yahya presented as a quiet, reserved teen who was worked hard throughout the evaluation. Overall, Yahya performed in the below average range on a measure of intellectual functioning. His self-reported adaptive functioning is exceptionally low. Taken together with reported difficulties in completing activities of daily living by both Yahya and his mother, he meets criteria for a diagnosis of **Mild Intellectual Disability**. At this time, Yahya does not meet criteria for a diagnosis of autism spectrum disorder. While he shows delays in social skills, these are likely secondary to his overall cognitive delays.

Additionally, Yahya and his mother are endorsing that he experiences substantial symptoms of anxiety including general anxiety and worries, fear of separating from his parents/independence, and social anxiety. Based upon there report, Yahya meets criteria for a diagnosis of **Generalized Anxiety Disorder (GAD)**.

While the etiology of Yahya's intellectual disabilities is unclear, medical records indicate that he experiences several bone development disorders. Taken together, there is a strong possibility that there is an underlying medical or genetic condition that is contributing to his physical and cognitive development. Specific recommendations based on these findings is included at the end of this report.

Background

Yahya's mother expressed ongoing concerns about cognitive, social, and emotional functioning. Yahya and his mother endorsed that he has difficulties with focusing, being easily distracted, avoiding tasks requiring mental effort, forgetting school assignments, seeming not to listen, and losing/misplacing items. Yahya endorsed difficulties with organization. His mother did not have concerns for his memory, but Yahya reported that he feels his memory is shorter compared to others. He also noted doing better when remembering visual information. Yahya spoke his first words at about 1.5 years old and used phrase speech at 2 years old. His mother reported that he first learned Arabic and speaks this at home. He started learning English at 7 years old while living in Egypt. Yahya reportedly has difficulty with putting thoughts together before speaking. His mother reported that he experienced substantial delays in his motor skills, and he was in physical therapy until 1st grade. She reported having to complete many daily activities for Yahya until he was about 10 years old. He is not currently in physical therapy, occupational therapy, or speech therapy. Yahya's mother reported that he does not complete activities of daily living on his own and frequently needs reminders to care for himself. He relies on his family for instrumental activities of daily living (e.g., handling finances, preparing food, and using transportation).

Emotionally, Yahya described his general mood as "positive." He did not endorsed any depression or sadness. His mother reported that he has periods where he will become easily upset and be in a bad mood for a couple hours. She reported that this occurs at least weekly, and it is easier to keep him comfortable instead of upsetting him. Yahya reported that he feels anxious when in the car and when presenting. His mother

reported that he is anxious in other situations including social situations. Yahya did not endorse any alcohol or substance use, hallucinations, or thoughts of suicide or self-harm. He is not working with a therapist.

Socially, Yahya reported that he does not have many friends. He enjoys playing games and talking with friends. His mother reported that he has trouble understanding how others feel and adjusting his behavior. His mother reported that some individuals befriended Yahya and were later discovered to have committed a serious crime. His mother expressed concerns that he will be taken advantage. She reported that he only recently started seeking out peers to interact with and previously tended to play alone. He has a history of parallel play and did not engage in imaginative play.

Yahya has a history of strong interests but does not stick with the same one for long. Previous examples include graphic design, biology, and architecture. No history of repetitive movements was reported. Yahya often struggles to adapt to changes in routine. Both his mother and him reported that he struggles with flexibility and will be less comfortable when this occurs. Yahya is sensitive to loud noises and insensitive/less reactive to pain.

Yahya is currently in 12th grade at Fairfield High School. Yahya reported that school is going well. He does not receive any formal supports. His mother reported that she frequently talks with his teachers and ensures that he supported in class. His mother reported that he tends to do better with math and has difficulty with writing out his thoughts. While he is better at math Yahya needs more time compared to peers regardless of academic activity. His mother reported that she used to sit next to him regularly and help him work through his schoolwork. She would also simplify lessons to help him with understanding. Yahya previously received private tutoring.

Medically, he is described as generally healthy with no major injuries, illnesses, surgeries, or hospitalizations reported. Yahya's medical record indicates that he has scoliosis, bilateral hip pain, bilateral brachydactyly of toes, transverse maxillary hypoplasia, and joint hypermobility. His mother reported poor diet and that he tends to eat more snacks and then eat poorly during meals. Yahya is farsighted but does not like wearing his classes. No sleep or hearing concerns were reported. He is not currently taking any medication.

Yahya currently lives with his parents and siblings. His mother is a stay at home mother. His father works as a computer engineer. Family history is remarkable for ADHD, heart disease, and anemia.

Behavioral Observations/Mental Status
Yahya presented as a quiet, reserved teen. Yahya and Dr. Haak wore face masks due to COVID-19 and hospital protocol. During the evaluation, he was socially engaged and talked about growing up and his interests. Yahya exhibited appropriate eye contact and use of gestures to communicate. His mood was euthymic with a broad range of affect. Speech was understandable and appropriate in volume. He was able to understand and follow directions and social chit chat. His expressed thoughts were coherent and task related. He was inattentive

at times, including making attention-to-detail errors. He exhibited good frustration tolerance and worked hard throughout the evaluation.
Suicide Risk Screening: None reported by Yahya or his mother.

Tests Administered

- Wechsler Adult Intelligence Scale - Fourth Edition (WAIS-IV)
- Autism Diagnostic Observation Schedule - Second Edition (ADOS-2)
- Wide Range Achievement Test - Fifth Edition (WRAT-5)
- Repeatable Battery for the Assessment of Neuropsychological Status (RBANS) - Update

Questionnaires

- Adaptive Behavior Assessment System - Third Edition (ABAS-3)
- Beck Depression Inventory - Second Edition (BDI-2)
- Behavior Assessment Scale for Children - Third Edition (BASC-3)
- Multidimensional Anxiety Scale for Children - Second Edition (MASC-2)
- Childhood Autism Rating Scale - Second Edition (CARS-2)

Test Results and Interpretation
Yahya was oriented to date, day of the week, city he was in performed in the below average range overall on a measure of intellectual functioning. He was not oriented to location or time of day. In that context, he demonstrated below average overall intellectual functioning with relative strength in his low average nonverbal reasoning compared to below average verbal reasoning. Working memory was exceptionally low with below average processing speed.

On a screening measure of cognitive functioning, Yahya performed in the exceptionally low range overall with a significant split in his exceptionally low language and attention performances compared to low average memory and average visuospatial skills. On memory tasks, Yahya demonstrated intact list and figure memory but struggled to learn and retain a short story.

Word reading accuracy was below average and at a mid 4th grade level.

Autism-Related Measures
Yahya was administered the ADOS-2 Module 4, a semi-structured measure of autism-related symptoms in adults with fluent speech. Yahya exhibited complex speech and used appropriate volume and intonation while speaking. No echoed or stereotyped speech was observed. He frequently offered information about his own thoughts, feelings, and experiences. He tended to respond appropriately to examiner's shared thoughts but did not spontaneously inquire about this information. Yahya shared information about nonroutine events that had occurred to him. He was able to elaborate on his thoughts and respond to the examiner though conversations tended to be less flexible than expected. He frequently used descriptive and informational gestures. He exhibited emphatic and emotional gestures though these were limited in frequency.

Yahya use appropriate eye contact and directed facial expressions appropriately toward the examiner. While he tended to coordinate these

with his spoken thoughts, he was less flexible in doing so. Yahya exhibited shared enjoyment in a number of activities during the ADOS-2. He exhibited some ability to describe his own emotions, though more limited than expected . He showed an understanding and recognition of emotions in others during various tasks. Yahya showed limited insight into a variety of social relationships. He provided indications of some responsibility for his actions though inconsistently across tasks. He made some attempts at gaining the examiner's attention, and these were generally effective and appropriate. He showed some responsiveness to the examiner's social initiations though these were somewhat limited. Overall, interaction was sometimes comfortable but not sustained.

Yahya exhibited some creativity across different tasks in the ADOS-2 though more limited in range. No sensory interests, repetitive movements, self-injurious behavior, excessive interests, or compulsions were observed during the ADOS-2. Overall, Yahya's presentation was consistent with a non-spectrum presentation.

The examiner completed a rating measure of autism spectrum symptoms based upon clinical history, performance-based measures, collateral report, and behavioral observations. Yahya exhibits minimal-to-no symptoms of ASD. This rating is consistent findings from the ADOS-2.

Adaptive Functioning and Emotional/Behavioral Rating Questionnaires
Yahya's mother completed a measure of general emotional and behavioral functioning. She endorsed borderline difficulties for Yahya related to leadership and completing activities of daily living.

Yahya completed a measure of his adaptive functioning. Based on his self-report, he exhibits exceptionally low overall adaptive functioning with exceptionally low conceptual, social, and practical skills. His mother partially completed a collateral report measure of Yahya's adaptive functioning. Her ratings of his functioning in different areas ranged from exceptionally low to below average.

Yahya is endorsing a mild level of depressive symptoms on a self-report measure. Both he and his mother completed a measure of anxiety symptoms. Both endorsed that Yahya is experiencing significant symptoms related to separation anxiety, generalized anxiety, and social anxiety. His mother also indicated that Yahya experiences clinically significant physical symptoms of anxiety including panic symptoms and tenseness and restlessness. Yahya also endorsed borderline difficulties related to obsessive thinking and hard avoidance.

Recommendations
Based on the results of the current evaluation, the following recommendations are provided:

1. Application for ID waiver services is strongly recommended.

2. Yahya would benefit from psychotherapy to help with anxiety symptoms. He would benefit from developing coping strategies and distress tolerance. Family involvement is important to ensure

that he is able to learn and implement skills from therapy.

3. Yahya's family is encouraged to share this report with treatment providers to aid in ongoing treatment planning. Given the evidence, he would benefit from a workup to rule out possible genetic or medical conditions that may be causing his presentation.

4. Yahya may benefit from going through the transition clinic through the Center for Disabilities and Development. This program is meant to help identify needs and resources for individuals who are preparing for life after high school. Areas of focus include employment, life skills, housing, resource identification, and self-advocacy.

5. Yahya is recommended to continue to attend all medical appointments with a family member to ensure understanding of treatments and medical needs.

6. For general everyday activities, Yahya should attempt to utilize compensatory strategies to help with organization and memory, such as use of an appointment calendar, reminder notes, prioritization lists, and alarms to remind him of important tasks. He may wish to work with his therapist or with an executive functioning coach on developing and implementing these skills.

7. Yahya's family is encouraged to consider guardianship or seeking power of attorney for him given his reliance on others for support in activities of daily living and decision making. Power of attorney would be utilized for major decisions including decisions related to healthcare and major financial decisions. Iowa Legal Aid has more information about guardianship including when it may be appropriate. https://www.iowalegalaid.org/resource/what-is-guardianship-and-conservatorship-1?utm_source=lh_rss#

8. Yahya will function best in an environment with a high level of structure and routine.

9. Yahya would benefit from a 504-Plan to support his learning. While there may not be time to implement these supports in high school, Yahya will continue to require and benefit from supports when he goes to college. Yahya should provide a copy of this report to his future college's resource center prior to matriculation in order to get accommodations in place prior to beginning his first semester. Dr. Haak is available for consultation in an effort to share and discuss current test results with school personnel. Recommendations for accommodations are outlined below.

<u>Accommodations</u>

a. Test Accommodations (should apply to district and state testing as well)
    - Given his learning needs and anxiety, which are likely to impact his test performances, Yahya should be given extended time on tests.
    - Yahya would benefit from taking tests in a quiet environment

free from distraction (small group setting) as needed.

Attention and Organization

a. Yahya will benefit from structured situations with clear rules for academic tasks, including what to do when, and frequent reinforcement. The more his environment is structured so that he knows what to expect and what to look for, the easier it will be for his to adapt to what is demanded and to incorporate it into his daily functioning.
b. Yahya should minimize distractions to the greatest extent possible when studying. It is recommended that, when possible, he not have his computer or other electronic devices near his while he is studying, as to prevent getting distracted by the internet and/or games.
c. Yahya should construct small notes on index cards that he can place at his workspace to help guide his attention, increase self-monitoring, organization, and on-task behavior. These notes should be few in number and brief in content. Yahya should be involved in making up the “reminder” index card so that he feels a sense of responsibility of what is expected of his throughout independent class work.
d. Whenever possible, Yahya should repeat and/or rephrase verbal and nonverbal directions. This strategy, which is called “think aloud,” is an effective way to build and foster his own inner speech. This will make it easier for his to understand and realize when he has questions and needs to seek help or clarification. Yahya should be required to ask for clarification or assistance as part of every assignment.

Study Skills

a. It is recommended that Yahya begin structuring his study time in order to increase his ability to concentrate. Yahya should develop clear and realistic goals for each study session (e.g., number of pages, number of problems, etc.), decide the order in which he will complete tasks, and plan a reward.
b. Having a specific, realistic goal for all study sessions is essential. Expecting to accomplish too much in the time you have leads to discouragement. Setting and achieving goals leads to a sense of accomplishment. This is also critical for reducing stress and meeting deadlines. Underestimating how long a task will take is likely to result in having less time to do another task, which can cause additional stress.
c. Yahya should learn time management strategies. he needs to learn to estimate how long assignments and projects will take, as well as how much time is required to study for an exam.
d. It is recommended that Yahya begin his study sessions with subjects he finds difficult or boring. Yahya’s ability to concentrate will be highest when he begins studying and the task is more likely to take less time if he is at his best while doing it.
e. Yahya should have a regular schedule in which he does homework at the same time, in the same place. his workspace should be quiet and contain all supplies necessary to complete assignments. Yahya should also use a timer to mark work periods and scheduled breaks, which should be taken regularly. Yahya may find it helpful to visit the website http://mytomatoes.com/ or app Focus Keeper to augment his studying. This study technique uses the “pomodoro technique,”

which highlights the importance of working for short periods of time and taking frequent breaks

f. Planning to reward yourself when you have finished your study session can be very effective for helping you accomplish the tasks you need to do, but only if restraint is used and the reward is not available until studying is completed for the day. Yahya should make a list of potential rewards prior to studying so that time is not spent thinking about potential rewards each day when planning a study period.

g. When memorizing facts, Yahya should break memorizing into short time periods. Reciting information aloud as he writes it can be helpful. Use of mnemonics, visualizing the information in his head (for material where this is appropriate), and focusing on keywords can all help the memorization process.

h. Although minimizing external distractions is often easier to do, Yahya might find that it is more difficult to minimize internal distractions (e.g., grumbling stomach, thinking about other things to do, or mind wandering in general). Use of self-talk can help in this capacity. For example, if Yahya's stomach starts to grumble, he might say to himself, "It's almost lunchtime and I'm hungry. I have to study for another 30 minutes before lunch. I'll ignore my stomach noises."

College Transition

a. When transitioning to college, Yahya and his family are encouraged to utilize the different resources provided at his school (e.g., student support services, learning support center, counseling and psychological services) in order to support his adjustment to college. Advanced preparation that enhances the familiarity of the environment (e.g., visiting his future college) and his support networks (e.g., connecting with other freshmen in summer) would be beneficial.

Additional Helpful References and Resources

1. Resources and information for Intellectual Disability
   a. Websites
      i. American Association on Intellectual and Developmental Disabilities (AAIDD; Information) https://aaidd.org/home
      ii. PACER (information, advocacy, and parent training) www.pacer.org
   b. Books for Intellectual Disability
      i. *Steps to Independence* by Bruce Baker & associates (1997, Paul Brookes Publishers). Outlines a practical and scientifically-based approach to teaching basic self-care and community-living skills, as well as techniques for managing problem behaviors.
2. Information regarding depression and anxiety
   a. Books for emotional management
      i. *The Happiness Trap: How to Stop Struggling and Start Living* by Russ Harris & Steven Hayes
      ii. *Get Out of Your Mind and Into Your Life for Teens: A Guide to Living an Extraordinary Life* by Derek Ciarrochi, Louise Hayes, and Ann Bailey.
      iii. *SOS Help For Emotions: Managing Anxiety, Anger, And Depression* by Lynne Clarke
      iv. *Adolescent Depression: A Guide for Parents* by Francis

Mondimore
v. *When Nothing Matters Anymore: A Survival Guide for Depressed Teens* by Bev Cobain
vi. *Feeling Good: The New Mood Therapy* by David Burns provides helpful strategies to control depression.
vii. *The 10 Best-Ever Depression Management Techniques* by Margaret Wehrenberg
viii. *The Stress Reduction Workbook for Teens: Mindfulness Skills to Help You Deal with Stress* by Gina M. Biegel.
ix. *The Anxiety Workbook for Teens: Activities to Help You Deal with Anxiety and Worry* by Lisa Schab.
x. *The 10 Best-Ever Anxiety Management Techniques* by Margaret Wehrenberg
xi. *The Imp of the Mind: Exploring the Silent Epidemic of Obsessive Bad Thoughts* by Lee Baer
xii. *Overcoming Unwanted Intrusive Thoughts: A CBT-Based Guide to Getting Over Frightening, Obsessive, or Disturbing Thoughts* by Sally Winston & Martin Seif

b. Information and resources regarding mood problems
i. The National Alliance on Mental Health (NAMI) has information on anxiety, and depression as well as support services and local support groups (nami.org)
ii. *Anxiety and Depression Association of America* provides several resources regarding treatment of depression on their website: www.adaa.org
iii. The website of the American Academy of Child and Adolescent Psychiatry (www.aacap.org) provides helpful information related to different mood disorders among pediatric populations.
iv. The National Institute of Mental Health (NIMH) has information about mood problems among children and teenagers

3. Study Skills:
a. Books:
i. *How to Study in College, 7th Edition* by Walter Pauk.
ii. *Advanced Skills for School Success for Grades 7 – 12* available through Curriculum Associates Press
iii. *Becoming a Master Student* (2000) by David Ellis, available through Houghton Mifflin College Press

4. Information about advocacy and educational rights:
a. Books:
i. *A Guide to Special Education Advocacy* by Matt Cohen
ii. *From Emotions to Advocacy* by Pam and Peter Wright (see the author's website www.wrightslaw.com).
b. Websites:
i. www.parentcenterhub.org/ (Center for Parent Information and Resources)
ii. To learn more about his educational rights under the American Disabilities Act (www.frcd.org).

It has been a pleasure working with Yahya and his family. If you have any further questions or need additional assistance in implementing any of the above recommendations, please do not hesitate to contact us at 319-356-1446.

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Christopher L. Haak, Ph.D, HSP
Lifespan Neuropsychologist
Licensed Clinical Psychologist
Center for Disabilities and Development
University of Iowa Children's Hospital

Total time of 201 minutes was spent in Psychological or Neuropsychological Test administration and scoring.
CPT 96136 first 30 minutes (Qty 1)
CPT 96137 each additional 30 minutes (Qty 6)

Total time in Psychological testing evaluation services was 173 minutes, including interactive feedback to the family, record review, and report documentation.
CPT 96130 first hour (Qty 1)
CPT 96131 each additional hour (Qty 2)

A Psychiatric Diagnostic Evaluation was completed
CPT 90791

Encounter Diagnoses

	ICD-10-CM
1. ADHD (attention deficit hyperactivity disorder) evaluation	Z13.39
2. Learning difficulty	F81.9
3. Inadequate social skills, not elsewhere classified	Z73.4

Summary of Neuropsychological Test Results:

The following standardized scores are presented in the data summary sheet below. These scores indicate performance compared to other same aged children. Standard scores have an average range of 85-115. Scaled scores have an average range of 7-13. T-scores have an average range of 40 – 60. Percentile ranks show how well a child performed compared to a group of children the same age from across the United States. For example, if the percentile is 45, it means that he/she ranked at or above 45 out of 100 students his/her age, which would be considered solidly average. Scores should not be interpreted without the context of the accompanying report.

Wechsler Memory Scales-Third Edition (WMS-III)

Memory Subtests	Percentile	Description
Information and Orientation	3	Below Average

Wechsler Adult Intelligence Scale- Fourth Edition (WAIS-IV)

Intelligence/Index Scales	Standard	Percentile	Description
Full Scale IQ	71	3	Below Average
Verbal Comprehension	74	4	Below Average
Perceptual Reasoning	84	14	Low Average
Working Memory	66	1	Exceptionally Low
Processing Speed	74	4	Below Average
Core Intellectual Subtests	**Scaled**	**Percentile**	**Description**
Verbal Comprehension			
Similarities	6	9	Low Average
Vocabulary	5	5	Below Average
Information	5	5	Below Average
Perceptual Reasoning			
Block Design	7	16	Low Average
Matrix Reasoning	8	25	Average
Visual Puzzles	7	16	Low Average
Working Memory			
Digit Span	5	5	Below Average
Arithmetic	3	1	Exceptionally Low
Processing Speed			
Symbol Search	6	9	Low Average
Coding	4	2	Below Average

Repeatable Battery for the Assessment of Neuropsychological Status (RBANS)

Subtests	Score	Percentile	Description
Total Scale	**65**	**1**	Exceptionally Low
Immediate Memory	**82**	**12**	Low Average
List Learning	12	75	High Average
Story Memory	2	<1	Exceptionally Low
Visuospatial/Constructional	**48**	**<1**	Exceptionally Low
Figure Copy	10	50	Average
Line Orientation		26-50	Average
Langauge	**51**	**<1**	Exceptionally Low
Picture Naming		<2	Exceptionally Low
Semantic Fluency	2	<1	Exceptionally Low
Attention	**48**	**<1**	Exceptionally Low
Digit Span	4	2	Below Average
Coding	1	<1	Exceptionally Low
Delayed Memory	**86**	**18**	Low Average
List Recall		>75	High Average
List Recognition		51-75	Average
Story Recall	5	5	Below Average
Figure Recall	10	50	Average

Wide Range Achievement Test -Fifth Edition (WRAT-5)

Subtests	Standard	Percentile	Description
Word Reading	75	5	Well Below Average

ADOS-2 Module 4 Algorithm for DSM-V Diagnosis

Description	
Classification	Non-Spectrum

Childhood Autism Rating Scale-2nd Ed., Standard Version (CARS2-ST)

Classification	Raw
Total Score	23.0
minimal-to-no-symptoms of ASD	

Beck Depression Inventory - Second Edition (BDI-II)

Raw

BDI-II	15

Manifest Anxiety Scale for Children - Second Edition (MASC-2)

	Self-Report	Mother
MASC-2 Total Score	**68***	**72****
Separation Anxiety/Phobias	79**	71**
GAD Index	62*	82**
Social Anxiety Total	61*	69*
Humiliation/Rejection	61*	61*
Performance Fears	58	75**
Obsession/Compulsions	64*	Too many missing items
Physical Symptoms Total	55	90**
Panic	58	90**
Tense Restless	50	70**
Harm Avoidance	60*	40

*At-Risk **Clinically Significant T-Score

BASC-3 Parent Rating Scales -Adolescent (PRS-A)

	Mother
Externalizing Problems	**45**
Hyperactivity	39
Aggression	54
Conduct Problems	44
Internalizing Problems	**42**
Anxiety	37
Depression	43
Somatization	50
Behavioral Symptoms Index	**50**
Atypicality	53
Withdrawal	54
Attention Problems	59
Adaptive Skills	**42**
Adaptability	59
Social Skills	41
Leadership	39*
Activities of Daily Living	32*
Functional Communication	47

*At-Risk **Clinically Significant T-Score

Adaptive Behavior Assessment System-Third Edition - Self-Report

Composites	Standard	Percentile
General Adaptive Composite	54	<1
Conceptual Skills	54	<1
Social Skills	68	2
Practical Skills	52	<1
Domains	**Scaled**	**Percentile**
Communication	5	5
Functional Academics	1	<1
Self-Direction	1	<1
Leisure	4	2
Social	4	2
Community Use	1	<1
Home Living	3	1
Health and Safety	1	<1
Self-Care	1	<1

Adaptive Behavior Assessment System-Third Edition - Mother's Report

Domains	Scaled	Percentile
Functional Academics	5	5
Social	5	5
Community Use	4	2
Home Living	1	<1

Electronically signed by Chris Haak at 3/18/2022 9:14 AM

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